

Premier — Semaglutide / Glycine

GLP-1 Provider Reference

Weight Loss Program · KORB Health Group · korb-glp1-data.js v1.2 · generated 2026-08-09

Everything needed to prescribe Premier semaglutide / glycine, complete on its own. Values are copied literally into the Tebra Compound section — do not paraphrase, and do not adjust quantity, refill or days supply.

Premier Pharmacy at a glance

Status	Active
Preferred states	TX, NV, AZ, FL
Ships to	AZ, CO, CT, DC, DE, FL, GA, IL, KS, KY, LA, MD, ME, MI, MO, MS, MT, NC, ND, NE, NJ, NM, NV, NY, OH, OK, OR, PA, RI, SD, TN, TX, UT, VA, VT, WI, WV, WY
CANNOT ship to	AK, AL, AR, CA, HI, IA, ID, IN, MA, MN, NH, SC, WA
Order via	Tebra Compound
Billing	Bill to KORB Health Group, ship to patient
Dispensing address	Premier Pharmacy, 2425 Babcock Rd, Ste 108A, San Antonio, TX 78229

Before you prescribe

- All Premier programs offer a 4-week and an 8-week option — semaglutide, semaglutide with glycine, and tirzepatide alike.
- All Premier compounds carry a 90-day BUD as of 2026-08. There is no longer a difference between the glycine and non-glycine products on this.
- 8-week programs ship the full eight weeks of medication and supplies in a single initial shipment. No refill and no second shipment.
- Vials remain 28 days from first use regardless of BUD. On lower doses this produces overage. Counsel the patient to discard at 28 days.

Premier — Semaglutide / B-12 / Glycine

Formulation: Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL

Route: subcutaneous · **Frequency:** once weekly · **Order via:** Tebra Compound

Same for every dose

Drug Formulation	Semaglutide/B-12/Glycine 3mg/0.5mg/5mg per mL inj
Select	Allow Substitution
Unit	ml
Reason for Compounding	N/V mitigation & flexibility
Pharmacy Instructions	Bill to KORB Health Group and ship to the patient. Custom Rx for N/V mitigation, dosing flexibility, and added B-12 and glycine.

4-week program

Dose	Name	Patient Instructions	Qty	RF	Days
0.3 mg	PREMIER – Glycine Semaglutide 0.3 mg – 4-Week Supply	GLYCINE – INJECT 0.3 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS	1	0	28
0.6 mg	PREMIER – Glycine Semaglutide 0.6 mg – 4-Week Supply	GLYCINE – INJECT 0.6 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS	1	0	28
1.2 mg	PREMIER – Glycine Semaglutide 1.2 mg – 4-Week Supply	GLYCINE – INJECT 1.2 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS	2	0	28
1.8 mg	PREMIER – Glycine Semaglutide 1.8 mg – 4-Week Supply	GLYCINE – INJECT 1.8 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS	3	0	28
2.7 mg	PREMIER – Glycine Semaglutide 2.7 mg – 4-Week Supply	GLYCINE – INJECT 2.7 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS	4	0	28

8-week program

Dose	Name	Patient Instructions	Qty	RF	Days
0.3 mg	PREMIER – Glycine Semaglutide 0.3 mg – 8-Week Supply	Maintenance – GLYCINE – INJECT 0.3 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS	2	0	56
0.6 mg	PREMIER – Glycine Semaglutide 0.6 mg – 8-Week Supply	Maintenance – GLYCINE – INJECT 0.6 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS	2	0	56
1.2 mg	PREMIER – Glycine Semaglutide 1.2 mg – 8-Week Supply	Maintenance – GLYCINE – INJECT 1.2 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS	4	0	56
1.8 mg	PREMIER – Glycine Semaglutide 1.8 mg – 8-Week Supply	Maintenance – GLYCINE – INJECT 1.8 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS	5	0	56
2.7 mg	PREMIER – Glycine Semaglutide 2.7 mg – 8-Week Supply	Maintenance – GLYCINE – INJECT 2.7 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS	8	0	56

Vials dispensed

Dose	Units	Concentration	4-week	8-week
0.3 mg	10 u	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml vial	1 ml x 2 vials
0.6 mg	20 u	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml vial	1 ml x 2 vials
1.2 mg	40 u	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml x 2 vials	1 ml x 4 vials
1.8 mg	60 u	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml x 3 vials	1 ml x 5 vials
2.7 mg	90 u	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml x 4 vials	1 ml x 8 vials

Pricing and charge codes

Provider and internal only. Never quote a price to a patient from this document without confirming with Operations. Includes: Telehealth visit, medication, shipping and supplies.

Semaglutide — flat, not dose-tiered

Program	Price	Charge code
4-week — All others	Varies	Operations provides at scheduling
4-week — Discounted rate (Corporate partners, Friends and family, Military)	\$199	Operations provides at scheduling
4-week — Full price — website rate	\$269	FITSema001
8-week	\$349	FITSemaMNT

- No discounted rate on 8-week. Corporate, friends and family, and military patients all pay \$349 for 8-week. The \$199 rate is 4-week only.
- **Why most 4-week codes are not listed:** Many codes per organization for tracking; partner contributions vary and change often. Operations, at the time the patient is scheduled.

If the patient moves state

This formulation is only dispensed by Premier Pharmacy, so the states below are the complete list where it is available.

Premier Pharmacy ships to	AZ, CO, CT, DC, DE, FL, GA, IL, KS, KY, LA, MD, ME, MI, MO, MS, MT, NC, ND, NE, NJ, NM, NV, NY, OH, OK, OR, PA, RI, SD, TN, TX, UT, VA, VT, WI, WV, WY
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Moving outside these states

The glycine formulation is only available from Premier. A patient moving to a state Premier cannot ship to must move to another pharmacy and switch to semaglutide WITHOUT glycine — there is no glycine equivalent elsewhere.

- Established patients normally stay with their current pharmacy unless there is a clinical or supply reason to move, or the new state is one this pharmacy cannot serve.

Is this the right patient

Program: **Metabolic Health & Weight Loss Program** · Visit cadence: **Every 4 to 8 weeks**

Indications

• Weight loss
• Medically driven weight loss
• Increased energy
• Better sleep
• Improved mental health
• Improved cardiovascular health

Identifying the appropriate candidate

Body mass index	BMI greater than 25 is overweight · BMI greater than 30 is obese
FDA-approved uses	• Weight loss • Type 2 diabetes • Prevention of cardiovascular death, myocardial infarction and stroke in adults with established cardiovascular disease who are overweight or obese
Diabetic and pre-diabetic patients	KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Program length and follow-up

INTERNAL ONLY. Internal and provider-facing only. Do not put this rule in a patient handout, the website, a pricing flyer, or any Circle post. A patient should not arrive expecting to graduate to 8-week.

Start rule	Every patient starts on the 4-week program. No exceptions.
Move to 8-week	A patient may be offered the 8-week program once they are stable at the dose they have landed on after escalation.
Who decides	Provider — provider-driven throughout. The 8-week program is an offer the provider extends, not a milestone the patient reaches or requests. Escalation, holding, reducing and weaning off are all provider discretion.
Visit cadence	4-week: Monthly provider visit. 8-week: Provider visit every 8 weeks. Either a video visit or an asynchronous visit. Both are acceptable for 4-week and 8-week follow-ups.

Weaning	Dose reduction and weaning off therapy are entirely at provider discretion. No fixed taper schedule is defined at the program level.
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Reasons a patient stays at 4-week

• Cost — the 4-week program is the lower per-visit outlay and carries the discounted rate.
• Accountability — some patients want to see the provider monthly.
• Clinical — the provider judges that side effects are not adequately controlled and the patient needs to be seen more often.

Agent monograph — Semaglutide — GLP-1 receptor agonist

Definition	Long-acting glucagon-like peptide-1 (GLP-1) receptor agonist. A 31-amino-acid analogue with roughly 94% homology to human GLP-1, acylated with a C18 fatty diacid that binds albumin and extends the half-life to about one week, allowing once-weekly dosing. FDA-approved as Ozempic and Rybelsus for type 2 diabetes and as Wegovy for chronic weight management.
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Mechanism

• Agonises the GLP-1 receptor.
• Glucose-dependent insulin secretion from pancreatic beta cells.
• Suppression of inappropriate glucagon release.
• Slowed gastric emptying, which prolongs satiety.
• Direct action on hypothalamic appetite centres, principally the arcuate nucleus, reducing hunger and energy intake.
• Insulin release is glucose-dependent, so hypoglycaemia risk is low as monotherapy.

Clinical evidence

• Strong randomised evidence — for the BRANDED product.
• STEP 1: mean weight loss near 15% of body weight at 68 weeks, against roughly 2.4% on placebo.
• SELECT: significant reduction in major adverse cardiovascular events in adults with overweight or obesity and established cardiovascular disease, without diabetes.
• Weight regain is common after discontinuation. Raise this at the start rather than letting it become a later surprise.

Compounded preparation

KORB dispenses a COMPOUNDED preparation, not the FDA-approved branded product. The molecule is approved; this specific formulation is not, and compounded products are not reviewed by the FDA for safety, efficacy or quality. Counsel accordingly and document that the distinction was explained.

Contraindications

Absolute — do not initiate	Cautions — evaluate before initiating
• Personal or family history of medullary thyroid carcinoma (MTC)	• History of pancreatitis — evaluate carefully before initiating
• Multiple endocrine neoplasia syndrome type 2 (MEN2)	• Gastroparesis or other significant gastrointestinal motility disorder
• Known hypersensitivity to semaglutide or any component of the formulation	• Active gallbladder disease or prior gallbladder-related surgical complications
• Pregnancy, breastfeeding, or planning pregnancy	• Severe renal impairment (eGFR below 30 mL/min/1.73 m ²)
	• Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
	• History of suicidal ideation or active depression — no causal link has been established in regulatory review, but monitor and ask

Medication interactions

- Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
- Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
- Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
- ANAESTHESIA AND PROCEDURES: delayed gastric emptying raises aspiration risk under sedation. Anaesthesia guidance generally advises holding weekly GLP-1 therapy before an elective procedure. Tell the patient to disclose GLP-1 use to any surgeon, proceduralist or anaesthetist.
- Other GLP-1 receptor agonists: do not combine. Concomitant use is not recommended.

Side effects

Common	Less common
• Pain, redness or swelling at the injection site	• Pancreatitis
• Nausea and vomiting	• Gastroparesis
• Diarrhea	• Bowel obstruction
• Stomach pain	• Gallstone attacks
• Constipation	
• Low appetite	
• Headaches	
• Dizziness	

Monitoring

■ Weight and BMI at each visit	■ Right-upper-quadrant pain — assess for gallbladder disease
■ Ask about blood pressure — monitored by the PCP, not measured by KORB	■ Mood and mental health
■ Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB	■ Lean mass preservation — protein intake and resistance training
■ Gastrointestinal tolerance, especially through dose escalation	■ Injection site and adherence
■ Abdominal pain that is severe or radiates to the back — assess for pancreatitis	

Patient counseling

“Semaglutide works on the same pathway as a hormone your body already makes. It slows how quickly your stomach empties and reduces appetite signalling, so you feel full sooner and stay full longer. Nausea is the most common side effect and is usually worst in the first days after a dose increase. This is a compounded preparation rather than the brand-name product. Weight tends to come back if the medication stops, so we should talk about this as a long-term plan rather than a short course. Tell any surgeon or anaesthetist that you are taking this.”

- Gastrointestinal side effects are most common during dose escalation.
- Slower titration or holding at the current dose may improve tolerability.
- Instruct the patient to report persistent, severe or worsening symptoms.
- New or severe symptoms may require dose adjustment, temporary hold, or discontinuation.
- Vials are good for 28 days from first use regardless of the pharmacy BUD. Counsel the patient to discard at 28 days even if medication remains.

Chart attestation

Patient counseled on semaglutide as a GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, the need to disclose GLP-1 use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

Primary	Secondary
E66.9 — Obesity, unspecified	Z68.— — Body mass index (add the matching BMI code)
E66.01 — Morbid (severe) obesity due to excess calories	Z71.3 — Dietary counseling and surveillance
E66.3 — Overweight	Z72.3 — Lack of physical exercise

Escalation and discontinuation

Hard stop — discontinue immediately	Flag and reassess
• Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate. • Hypersensitivity or anaphylaxis-type reaction. • Pregnancy identified or confirmed. • Medullary thyroid carcinoma or MEN2 identified at any point. • Persistent severe vomiting with dehydration or acute kidney injury.	• Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward. • Right-upper-quadrant pain, or gallstone symptoms. • Rapid or excessive weight loss, or loss of lean mass. • New or worsening depression, or any mention of suicidal thoughts. • Worsening retinopathy in a diabetic patient during rapid glycaemic improvement. • Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely. • Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

• GLP-1 receptor agonists are not currently on the WADA prohibited list, so the athlete hard stop used in the Functional Health peptide program does not apply to this program. Confirm current WADA status before treating a tested competitive athlete.

Questions and escalation

- Pharmacy or shipping issues — Operations
- Charge codes and billing — Operations
- Clinical protocol questions — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute a pharmacy the tool marks as unavailable
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without confirming with Operations